

All MCQs

1. ***Regarding oropharyngeal functions the following answer is incorrect:**
 - a. Conduit passage of air or food
 - b. Helps in pharyngeal phase of deglutition
 - c. Provide local defense and immunity against harmful intruders into the air and food passage
 - d. Forms part of the vocal tract for certain speech sounds
 - e. Helps in appreciation of smell
2. ***The most common cause of bilateral recurrent laryngeal nerve paralysis is**
 - a. Laryngeal trauma
 - b. Surgical iatrogenic damage in case of total thyroidectomy
 - c. A tumor mass expanding in upper part of mediastinum
 - d. Toxic polyneuropathy of recurrent laryngeal nerves
 - e. Acute subglottic laryngitis
3. ***The most important symptom in otosclerosis is**
 - a. Vertigo
 - b. Conductive hearing loss
 - c. Sensorineural hearing loss
 - d. Aural fullness
 - e. Tinnitus
4. ***The election treatment in cholesteatoma is**
 - a. Antibiotherapy
 - b. Chemotherapy
 - c. Radiotherapy
 - d. Surgery
 - e. Wait and scan policy
5. ***The nasal septum divides the nose into**
 - a. Superior and inferior nasal cavity
 - b. Right and left nasal cavity
 - c. Lateral and medial cavity
 - d. Four cavities 2 superior and 2 inferior
 - e. Four cavities 2 medial and 2 lateral
6. ***Maxillary sinus**
 - a. Opens into posterior part of the ethmoidal infundibulum into the middle meatus
 - b. Opens into the superior meatus
 - c. Its innervation is from the CN IX
 - d. It is the smallest sinus
 - e. Has a 5 ml capacity
7. **Relations of external acoustic meatus are**
 - a. Superiorly – middle cranial fossa
 - b. Posteriorly – mastoid air cell and facial nerve
 - c. Inferiorly – parotid gland
 - d. Anteriorly – temporo-mandibular joint
 - e. Superiorly – mastoid air cells and facial nerve
8. **The Eustachian (pharyngotympanic) tube**
 - a. Connects the tympanic cavity with the nasopharynx
 - b. Bony part – lat 1/3
 - c. Fibrocartilaginous part – medial 2/3
 - d. Torus tubaris at the pharyngeal end
 - e. Zygomatic cells in the middle part

9. Ossicles of the middle ear are

- a. Malleus
- b. Tensor tympani
- c. Incus
- d. Stapes
- e. Stapedius

10. Predisposing factors for acute suppurative otitis media are

- a. Recurrent attacks of common cold, upper respiratory tract infections and exanthematous fever
- b. Infections of tonsils and adenoids
- c. Chronic tympanosclerosis
- d. Nasal allergy
- e. Tumours of nasopharynx, packing of nose or nasopharynx for epistaxis

11. Signs of acute mastoiditis are

- a. Mastoid tenderness
- b. Ear discharge: mucopurulent or purulent discharges
- c. Sagging of inferior wall
- d. Swelling over the mastoid
- e. Hearing loss – conductive type

12. Otosclerosis symptoms are

- a. Hearing loss – bilateral neurosensorial type
- b. Paracusis willisii – patient hears better in noisy environment than quiet surroundings
- c. Tinnitus
- d. Vertigo
- e. Monotonous well modulated soft speech

13. Differential diagnosis of Otosclerosis

- a. Serous otitis media
- b. Tympanosclerosis
- c. Attic fixation of malleus head
- d. Acoustic neuroma
- e. Ossicular discontinuity

14. Meniere's disease cardinal symptoms are

- a. Episodic vertigo
- b. Paracusis willisii
- c. Fluctuating hearing loss
- d. Tinnitus
- e. Sense of fullness in involved ear

15. Nerve supply of the larynx

- a. Motor: recurrent laryngeal nerve
- b. Sensory: internal laryngeal nerve (above vocal cord)
- c. Motor: recurrent laryngeal nerve (below vocal cord)
- d. Motor: recurrent laryngeal nerve (above vocal cord)
- e. Sensory: recurrent laryngeal nerve (below vocal cord)

16. Chronic hyperaemic laryngitis treatment

- a. Elimination of the infection of upper or lower respiratory tract
- b. Avoidance of irritating factors
- c. Surgery
- d. Voice rest and speech therapy
- e. Expectorants

17. Congenital lesions of the larynx are

- a. Laryngomalacia
- b. Subglottic haemangioma
- c. Laryngoesophageal cleft
- d. Pectus excavatum
- e. Laryngeal web

18. Treatment of polypoid corditis (Reinke's edema)

- a. Smoking cessation
- b. Surgery: cordectomy
- c. Antireflux medication
- d. Preoperative vocal therapy
- e. Surgery: vocal cord stripping

19. Benign tumours of the larynx are

- a. Squamous cell carcinoma
- b. Chondroma
- c. Haemangioma
- d. Granular cell tumour
- e. Glandular tumour

20. Etiology of larynx cancer

- a. Tobacco, alcohol
- b. Neck trauma
- c. Previous radiation
- d. Genetic factors
- e. Occupational exposure

21. Glottic cancer symptoms are

- a. Hoarseness is an early symptom
- b. Hoarseness is a late symptom
- c. Stridor and laryngeal obstruction in late stages
- d. Stridor and laryngeal obstruction in early stages
- e. Throat pain

22. Types of cordectomies

- a. type I - a subepithelial cordectomy, which is resection of the epithelium;
- b. type II - a subligamental cordectomy, which is a resection of the epithelium, Reinke's space and vocal ligament;
- c. type III - transmuscular cordectomy, which proceeds through the vocalis muscle;
- d. type IV - total cordectomy;
- e. type Va - extended cordectomy, which encompasses the contralateral vocal fold and the anterior commissure;
- f. type Vb - extended cordectomy, which includes the arytenoid;
- g. type Vc - extended cordectomy, which encompasses the subglottis;
- h. type Vd - extended cordectomy, which includes the ventricle.

23. Indications of tracheotomy are

- a. Respiratory obstruction
- b. Retained secretions
- c. Respiratory insufficiency
- d. Puberophonia
- e. Hoarseness

24. Types of tracheotomy are

- a. High tracheotomy; above the level of thyroid isthmus
- b. High tracheotomy: through the II and III rings
- c. Mid tracheotomy: through the II and III rings
- d. Mid tracheotomy: below the level of thyroid isthmus
- e. Low tracheotomy: below the level of thyroid isthmus

25. The frontal sinus drains

- a. In the superior meatus
- b. Into frontal recess (55%)
- c. Above but not into the infundibulum (30%)
- d. Into the infundibulum (15%)
- e. Above the bulla ethmoidalis

26. Functions of paranasal sinuses are

- a. Air conditioning of the inspired air: warming and humidifying
- b. Voice resonance
- c. Thermal insulators – protection of delicate structures in the orbit and cranium
- d. Nasal drainage
- e. Lighten the skull bones

27. Clinical features of acute maxillary sinusitis are

- a. Constitutional symptoms: fever, general malaise
- b. Headache
- c. Pain – over upper jaw, occasionally ipsilateral supraorbital region
- d. Tenderness (pressure over the anterior wall of antrum produces pain)
- e. Post nasal discharge (pus on the hard palate at anterior rhinoscopy)

28. Complications of acute maxillary sinusitis are

- a. Deep neck infection
- b. Subacute, chronic sinusitis
- c. Osteitis or osteomyelitis
- d. Orbital cellulitis or abscess, optic neuritis
- e. Cavernous sinus thrombosis

29. Clinical features of acute ethmoid sinusitis are

- a. Pain over the bridge of the nose, medial and deep to the eye; aggravated by movement
- b. Edema of lids
- c. Hoarseness
- d. Nasal discharge
- e. Swelling of the turbinate

30. Waldeyer's ring consists of

- a. Nasopharyngeal tonsil or the adenoids
- b. Palatine tonsils
- c. Lingual tonsils
- d. Gastric tonsils
- e. Mastoid tonsils
- f. Tubal tonsils (in Fossa of Rosenmüller)

31. The hypopharynx clinically subdivides into three regions

- a. Pyriform sinus (fossa) lies on the either side of the larynx expands from pharyngoepiglottic fold ... end of esophagus
- b. Postcricoid region: the part of the anterior wall of laryngopharynx between the upper and lower borders of cricoid lamina
- c. Posterior pharyngeal wall: extends from the level of the hyoid bone to the level of cricoarytenoid
- d. Vocal cord
- e. Ventricular fold

32. *Functions of the hypopharynx are

- a. Conduit for passage of air or food
- b. Breathing
- c. Help in pharyngeal phase of deglutition
- d. Air conditioning of the inspired air: warming and humidifying
- e. Forms part of vocal tract for certain speech sounds

33. Peritonsillar abscess (Quinsy) complications are

- a. Parapharyngeal abscess
- b. Larynx oedema
- c. Septicaemia
- d. Pneumonitis
- e. Spontaneous haemorrhage from carotid vein or jugular artery

34. On the lateral wall of the nasal cavities, we can find

- a. The middle meatus
- b. The nasal septum
- c. The inferior meatus
- d. The maxillary sinus ostium
- e. Adenoids

35. Nasal septum deviation can lead to

- a. Sinusitis
- b. Oral respiration
- c. Hypo-/anomia

36. * Regarding pharynx anatomy which of the following statements is incorrect:

- a) It extends from base of skull to inferior border of cricoid cartilage anteriorly and inf border of C6 posterior
- b) Widest portion (5cm) is at hyoid level
- c) Narrowest portion (1.5cm) is at its caudal end
- d) The pharynx is divided into 3 parts: naso-/ oro- and laryngopharynx
- e) The pharynx has more than 20cm in length

37. The structures of pharyngeal wall are the following, with one exception. Indicate the exception:

- a) Mucous membrane
- b) Pharyngeal aponeurosis
- c) Muscular fascia
- d) Buccopharyngeal fascia
- e) Periosteum

38. Waldeyer's ring is composed of the following elements except one

- a. Submaxillary glands
- b. Nasopharyngeal tonsil or the adenoids
- c. Palatine tonsils
- d. Tubal tonsils (in Fossa of Rosenmüller)
- e. Nodules (on post pharyngeal wall)

39. *Nasopharyngeal functions are following except one

- a. Conduit for air
- b. Ventilates the middle ear through Eustachian tube
- c. Resonance
- d. Passage for foods and liquids
- e. Drainage channel

40. Adenitis aural symptoms are

- a. Tubal obstruction
- b. Sinusitis
- c. Recurrent attacks of acute otitis media
- d. Nasal discharge
- e. Serous otitis media

41. Regarding Juvenile Nasopharyngeal Angiofibroma the following statements are correct

- a. Exact cause unknown
- b. Age and sex (exclusively in male in age group 40-60)
- c. Appears as progressive nasal obstruction and nasal speech
- d. Biopsy is associated with profound bleeding and therefore to be avoided
- e. Treatment is always surgical

42. Regarding nasopharyngeal cancer following statements are **correct**
- a. Age onset: 5th to 7th decades but may involve younger age groups
 - b. Cervical nodal metastasis may be the only manifestation
 - c. Otologic symptoms and sign are represented by conductive hearing loss, serous and suppurative otitis media
 - d. Ophthmo-neurological symptoms are always present at the beginning
 - e. Distant metastasis are into the eustachian tube
43. Which of the following are symptoms of acute tonsillitis?
- a. Sore throat
 - b. Difficulty in swallowing
 - c. Oral breathing
 - d. Fever
 - e. Ear ache
44. Which of the following are acute tonsillitis complications?
- a. Convergent strabismus
 - b. Rheumatic fever
 - c. Chronic tonsillitis
 - d. Acute glomerulonephritis
 - e. Subacute bacterial endocarditis
45. DD of acute tonsillitis is performed with
- a. Membranous tonsillitis
 - b. Diphtheria
 - c. Rhinitis
 - d. Vincent's angina
 - e. Infections mononucleosis
46. DD of acute tonsillitis is performed with
- a. Sinusitiy
 - b. Buccopharyngeal manifestations in agranulocytosis
 - c. Buccopharyngeal manifestations of leukaemia
 - d. Aphthous ulcers
 - e. Malignant tumour of the tonsil
47. Regarding hypopharyngeal malignant tumour which of the following statements are **correct**?
- a. Clinical features are represented by progressive dysphagia, malnutrition, weight loss, change of voice and dysphonia
 - b. Mirror examination may not be useful
 - c. The most frequent site of origin is subglottic region
 - d. Prognosis is poor
 - e. Surgery, radiotherapy and combined therapy
48. *The olfactory nerves (12-20) which pass through cribriform plate and end in the
- a. Frontal lobe
 - b. Temporal lobe
 - c. Olfactory lobe
 - d. Parietal lobe
 - e. Olfactory bulb
49. The rinophyma is a
- a. Benign slow growing mass
 - b. Slow growing malignant tumour
 - c. Appears to the hypertrophy of the sebaceous glands of the tip of the nose
 - d. Is treated with surgical excision or with laser Co2
 - e. Appears due to the hypertrophy of the perspiratory glands of the tip of the nose

50. The choanal atresia

- a. Can be uni-, bilateral, complete, incomplete, bony (90%) or membranous (10%)
- b. It is always bilateral, complete, bony or membranous
- c. It is an emergency in bilateral choanal atresia
- d. One of the diagnostic signs is the absence of air bubbles in nasal discharge
- e. An important diagnosis sign is the failure to pass a catheter from nose to pharynx

51. *A dermoid cyst is

- a. A malignant tumour
- b. Congenital tumour
- c. Infection
- d. Blood collection
- e. Sign of consumption

52. *Into the middle meatus open the following sinuses

- a. Maxillary, frontal and anterior ethmoidal
- b. Posterior ethmoidal sinuses
- c. Sphenoid sinus
- d. Cavernous sinus
- e. Sinus tympani

53. Septal haematoma

- a. Is a collection of blood under the perichondrium or periosteum of the nasal septum
- b. It can be caused by nasal trauma or septal surgery
- c. Treatment consists in aspiration with a wide bore sterile needle
- d. Incision and drainage in larger ones with bilateral nasal packing
- e. Is a collection of connective tissue

54. Nasal skin, which affirmations are correct?

- a. Thin skin and freely mobile, covers the nasal bones
- b. Thin skin with many sebaceous glands, covers nasal bone and lower lateral cartilages
- c. Thick adherent and rich in sebaceous glands skin cover the alar cartilages
- d. Thin and adherent skin covering the alar cartilages
- e. Thick skin with many sebaceous glands, covers nasal bone and upper lateral cartilages

55. Rhinoscleroma

- a. Etiology is a gram-negative bacillus – Klebsiella Rhinoscleromatis
- b. Etiology is a gram-positive – Staph aureus
- c. Clinical stages are: atrophic, granulomatous, cicatricial
- d. Certain diagnosis given by biopsy
- e. Treated with streptomycin and tetracycline

56. Treatment in allergic rhinitis the following are correct

- a. Avoidance of allergen
- b. Supraexposure to allergen
- c. Antihistaminics
- d. Corticosteroids (oral and topical)
- e. Radiotherapy

57. Symptoms of vasomotor rhinitis are

- a. Wheezing
- b. Excessive rhinorrhea
- c. Nasal obstruction
- d. Postnasal drip
- e. Cough

58. Local causes of epistaxis are

- a. Trauma / foreign bodies
- b. Infections
- c. Neoplasms of nose and paranasal sinuses
- d. Heat
- e. Cold

59. Posterior epistaxis

- a. Originates mostly from posterosuperior part of nasal cavity
- b. Occurs spontaneous often due to hypertension or arteriosclerosis
- c. Most common cause is trauma
- d. Mostly occurs in children or young adults
- e. Mild and easy to control

60. Complications of frontal sinusitis

- a. Orbital cellulitis
- b. Osteomyelitis of frontal bone
- c. Meningitis, extradural abscess or frontal lobe abscess
- d. Maxillary sinus necrosis
- e. Acute conjunctivitis

61. Correct affirmation about laryngomalacia

- a. Most common congenital laryngeal anomaly (50-75%)
- b. Most frequent cause of stridor in adults
- c. Male predominance 2:1
- d. Flaccidity of supraglottic laryngeal tissue
- e. Characterised by inward collapse of pharynx structures

62. Subglottic haemangiomas are

- a. Asymptomatic till 12 months
- b. 50% have associated cutaneous haemangiomas
- c. Stridor but a normal cry
- d. Reddish-blue mass below the vocal cords
- e. Biopsy – strongly indicated

63. *Epiglottitis is formed by

- a. Muscle
- b. Bone
- c. Fibrous cartilage
- d. Adipose tissue
- e. Skin

64. *Most common cause of bilateral recurrent laryngeal nerve paralysis is

- a. Laryngeal trauma
- b. Surgical iatrogenic damage in case of total thyroidectomy due to thyroid gland carcinoma
- c. Tumour mass expanding in upper part of mediastinum
- d. Toxic polyneuropathy of recurrent laryngeal nerves
- e. Acute subglottic laryngitis

65. Larynx functions are

- a. Phonation
- b. Respiration
- c. Inferior respiratory airway protection
- d. thoracic fixation
- e. Expectoration

66. Following are intracranial lesion of otitis media

- a. Extradural abscess
- b. Petrositis
- c. Meningitis
- d. Labyrinthitis
- e. Brain abscess

67. Abscesses in relation to mastoid infection are

- a. Postauricular abscess
- b. Zygomatic abscess
- c. Bezold abscess
- d. Petrositis
- e. Orbital cellulitis

68. Adenoiditis aural symptoms are not:

- a. Tubal obstruction
- b. Sinusitis
- c. Recurrent attacks of acute otitis media
- d. Nasal discharge
- e. Serous otitis media

69. *The epiglottis is formed by

- a. Muscle
- b. Bone
- c. Fibrous cartilage
- d. Adipose tissue
- e. Skin

70. Which of the following statements are correct:

- a. In Weber test the tuning fork is placed on the mastoid process
- b. In Weber test a patient with normal hearing localizes the tone either in the center of the head or equally in both ears
- c. In Weber test a patient with unilateral conductive hearing loss localizes the tone in the affected ear
- d. In Weber test a patient with neurosensorial hearing loss localizes the tone in the affected ear
- e. In Weber test a patient with neurosensorial hearing loss localizes the tone in the healthy ear

71. Differential diagnosis of acute tonsillitis is performed with

- a. Aphthous ulcers
- b. Malignant tumor of the tonsil
- c. Sinusitis
- d. Bucopharyngeal manifestations in Agranulocytosis
- e. Bucopharyngeal manifestations in Leukemia

72. Otosclerosis is characterized by

- a. Eustachian tube function is impaired
- b. Tinnitus
- c. Hyalinisation and calcification of subepithelial connective tissue, white chalky deposits in the promontory and ossicles
- d. Conductive hearing loss
- e. Paracusis willisii

73. Chronic non-specific laryngitis can NOT be:

- a. Catarale
- b. Caused by tbc
- c. Hypertrophic red and white type
- d. Atrophic
- e. Caused by syphilis

74. Waldeyer's ring is NOT composed of the following elements:

- a. Nasopharyngeal tonsil or the adenoids
- b. Palatine tonsils
- c. Lingual tonsils
- d. Gastric tonsils
- e. Mastoid tonsils
- f. Tubal tonsils (in Fossa of Rosenmüller)

75. In acute mastoiditis

- a. Treatment consists of stapedotomy
- b. Treatment consists of antibiotics
- c. Treatment consist of reconstructive surgery with hearing restoration by tympanoplasty
- d. Treatment consists of myringotomy
- e. Treatment consists of mastoidectomy

76. Abscesses in relation to mastoid infection are:

- a. Bezold
- b. Petrositis
- c. Zygomatic
- d. Orbital cellulitis
- e. Postauricular

77. Symptoms of acute tonsillitis are

- a. Oral breathing
- b. Difficulty in swallowing
- c. Fever
- d. Sore throat
- e. Earache

78. The hypopharynx is clinically subdivided into

- a. Vocal cord
- b. Ventricular fold
- c. Pyriform sinus
- d. Post-Cricoid area
- e. Posterior pharyngeal wall

79. Larynx benign tumours are

- a. Chondroma
- b. Osteoma
- c. Haemangioma
- d. Botriomycoma
- e. Papilloma

80. Congenital lesions of the larynx are

- a. Laryngomalacia
- b. Choanal atresia
- c. Laryngeal web
- d. Cleft palate
- e. Laryngocele

81. * The maxillary sinus

- a. Opens into the superior meatus
- b. Its innervations is from the IXth cranial nerve
- c. It is the largest sinus
- d. Opens into the superior meatus
- e. Has a 5ml capacity

82. Symptoms of vasomotor rhinitis are NOT

- a. Nasal obstruction
- b. Bronchospasm
- c. Postnasal drip
- d. Wheezing
- e. Ocular pruritis

83. Vocal nodules

- a. Are considered tumoral tissue reactions
- b. More frequent in male patients
- c. Are not favoured by voice overload
- d. Are surgically treated
- e. Voice rest is recommended

84. Complications of laryngotracheal trauma are represented by:

- a. Loss of taste
- b. Perichondritis/ laryngeal abscess
- c. Vocal cord paralysis
- d. Laryngeal stenosis
- e. Hiposmia

85. Laryngotracheal trauma can be represented by

- a. Haematoma and edema
- b. Othematoma
- c. Dislocation of cricoarytenoid joint
- d. Fractures of hyoid bones, thyroid and cricoids cartilages
- e. Fractures of upper tracheal rings

86. Following are intratemporal complications of otitis media

- a. Subdural abscess
- b. Facial paralysis
- c. Mastoiditis
- d. Meningitis
- e. Petrositis

87. *The structures of oropharyngeal wall are the following with one exception

- a. Mucous membrane
- b. Pharyngeal aponeurosis
- c. Muscular fascia
- d. Buccopharyngeal fascia
- e. Periosteum